FIGHTING THE SCARCITY

Chemotherapy drugs in short supply

CISPLATIN



Platinum-based chemotherapy drug that damages the DNA of cancer cells, preventing them from growing

Cancers treated: Head and neck, lung, bladder, as well as cervical, ovarian and testicular cancers

Approximate generic price: Rs 70–500 per vial*

CARBOPLATIN



Platinum-based chemotherapy drug similar to cisplatin but with a different side-effect profile

Cancers treated: Lung, breast, head and neck as well as ovarian and cervical cancers

Generic price: Rs 600–6,000 per vial

Other chemotherapy drugs that can be considered as substitutes

PACLITAXEL



A taxane chemotherapy drug. Taxanes are drugs that stop cancer cells from multiplying by disrupting microtubules needed for cell proliferation, thus slowing or stopping tumour growth

Cancers treated: Breast, lung ovarian and cervical cancers

Price: Rs 500–3,000 per vial

DOCETAXEL



Taxane chemotherapy medicine that slows/ stops cancer cell growth. Said to be more potent than Paclitaxel

Cancers treated: Breast, prostate, stomach and head and neck cancers

Price: Rs 1,500–8,000 per vial

DOXORUBICIN



An anthracycline chemotherapy drug. Anthracyclines damage cancer cells' DNA, preventing them from reproducing. Among the oldest chemo drugs

Cancers treated: Breast, lymphoma, sarcoma and ovarian cancers

Price: Rs 150–2,000 per vial

CYCLOPHOSPHAMIDE



A chemotherapy drug that damages cancer-cell DNA by attaching alkyl groups to them

Cancers treated: Breast, lymphoma, sarcoma and ovarian cancers

Price: Rs 150–2,000 per vial

**Prices are indicative and vary by strength, manufacturer and hospital procurement*

compounds and ifosfamide are widely used in ovarian, breast, mouth and throat, gynaecological and other cancers. A shortage can affect treatment across a wide spectrum of cancers."

Oncologists say that changing chemotherapy drugs often changes the side-effect profile or triggers a whole new set—not necessarily more severe—that requires monitoring, supportive medicines and counselling. For example, cisplatin carries a higher risk of kidney damage, hearing loss and severe nausea and vomiting, while carboplatin is more likely to suppress blood counts, increasing the risk of anaemia, infections and bleeding. Other chemotherapy drugs, such as paclitaxel and docetaxel, are associated with nerve damage in the hands and feet.

Alternative drugs, therefore, must be chosen only with utmost caution. "While doctors can often find alternative treatments, they are still alternatives. Whenever we are treating a patient with the hope of curing their cancer, even a small compromise is something we would prefer to avoid. Ensuring a steady supply of essential cancer medicines should be a priority," adds Dr Vishwanath.

MANAGING THE DEFICIT

The shortage has prompted hospitals to manage cancer medicines differently. Instead of waiting until stocks run low, they are now tracking inventories closely, coordinating with other hospitals and reaching out to multiple suppliers.

Experts say the episode should serve as a wake-up call. Platinum-based drugs have long been used in treating cancer, yet a disruption in raw material supply was enough to trigger a shortage. Better forecasting of demand, stronger coordination between manufacturers, hospitals and regulators and faster procurement mechanisms are essential to prevent such a situation.

The crisis has also reinforced the importance of patient counselling. Unlike common illnesses, cancer therapy is planned over months. Doctors say uncertainty over drug availability can heighten anxiety and lead patients to make impulsive decisions, making shared decision-making as important as the medicines themselves. "It is completely understandable for patients and families to feel anxious. However, a shortage does not mean that treatment will be less effective," says Dr Vishwanath. "Doctors should explain available options and what those choices mean for treatment. Honest conversations help reduce anxiety and allow patients to move forward with confidence."

Doctors are hopeful supplies will gradually improve as manufacturers respond to the revised price ceiling. For now, they agree on one message: patients should remain in close contact with their treatment team and seek help early if they are unable to access prescribed medicines. ■